

Public Health Emergency Operations Center
"COUSP DRC"

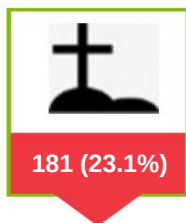
MVE-17 Incident Management System

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Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

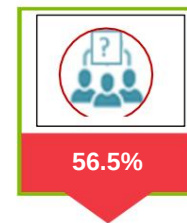
SitRep N°30/MVB_13/06/2026

Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (31)	• Ituri (20/36): Aru, Aungba, Bambu, Bunia, Damas, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia. • North Kivu (10/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi and Mabalako. • South Kivu (1/34): Miti-Murhesa
Reporting date	June 13, 2026
Publication date	June 14, 2026


 Total confirmed cases
for the 3 provinces

 Cumulative deaths
among confirmed cases
and case fatality rate

 Patients in
isolation -
hospitalization

 Cumulative number
of recoveries

 Suspected cases of
the day

 Contact tracing rates
for the 3 provinces

*Data is being harmonized to include confirmed cases still in the community as well as cases of tax evasion in the overall count.

Suspicious deaths occurring in the community are subject to investigation. further investigation with a view to possible reclassification (Non-cases, probable cases)

1. HIGHLIGHTS

Seventy -two (72) new confirmed cases, including 29 deaths, were reported during the day on June 13, 2026 in 2 provinces (Ituri: 56 and North Kivu: 16). These confirmed cases are distributed in the health zones of Mongbwalu (25), Bunia (13), Rwampara (6), Nyankunde (5), Komanda (3), Nizi (2), Damas (1) and Nia Nia (1) in the province of Ituri and the health zones of Butembo (11), Katwa (4) and Mabalako (1) for the province of North Kivu (currently being harmonized).

- Two new health zones affected, namely **Nia-Nia** in Ituri and **Mabalako** (North-Kivu).
- Five (5) patients (Rwampara: 3, Mongbwalu: 1 and Mambasa: 1) with Ebola virus disease have were **declared cured**.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola virus disease (EVD) outbreak, which was then raging in North Kivu. The current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a population

total estimated at nearly 15 million inhabitants, with massive internal displacements and cross-border flows to neighboring countries.

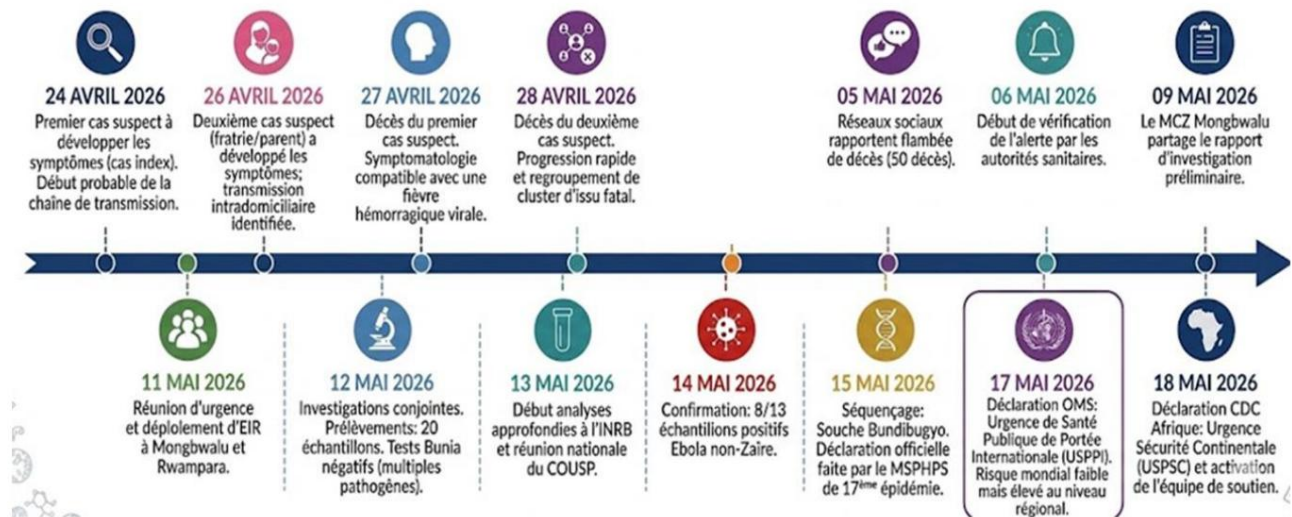


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of June 13, 2026

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	717	146	20.4%	20 out of 36 (55.5%)	56
North Kivu	62	34	54.8%	10 out of 34 (29.4%)	16
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	782	181	23.1%	31 out of 104 (29.8%)	72

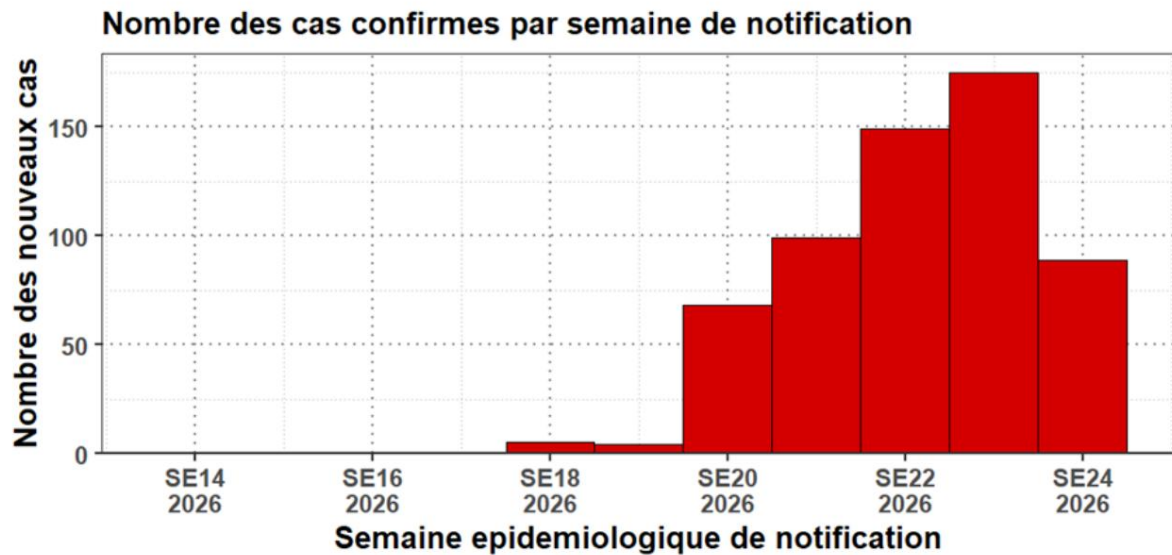
Two new health zones (ZS) were affected by the end of the day on June 13, 2026. This brings the total number of affected ZS to 31 out of 104 (section 3.1). Ituri province remains the epicenter, accounting for 91.8% of confirmed cases reported in the three affected provinces. North Kivu province remains the most affected in terms of case fatality rate (54.8%), while Ituri province shows a low case fatality rate (20.4%) despite the significant number of deaths recorded. South Kivu province has remained silent since May 26, 2026, the date on which its last confirmed case was reported.

3.2 Distribution by health zone (Cumulative as of 13/06/2026)

- **Province of Ituri (717 cases):** Bunia (212), Rwampara (149), Mongbwalu (164), Nyankunde (38), Nizi (11), Bambu (7), Komanda (6), Lita (6), Mangala (5), Kilo (4), Damas (4), Rimba (3), Aru (3), Aungba (2), Mambasa (2), Logo (2), Tchomia (2), Gety (1), Kambala (1) and Nia-Nia (1 case). (Ninety-four (94) additional cases are being ventilated).
- **North Kivu Province (62 cases):** Katwa (21), Butembo (18), Beni (14), Oicha (2), Kyondo (2), Kalunguta (1), Goma (1), Masereka (1), Vuhovi (1) and Mabalako (1).

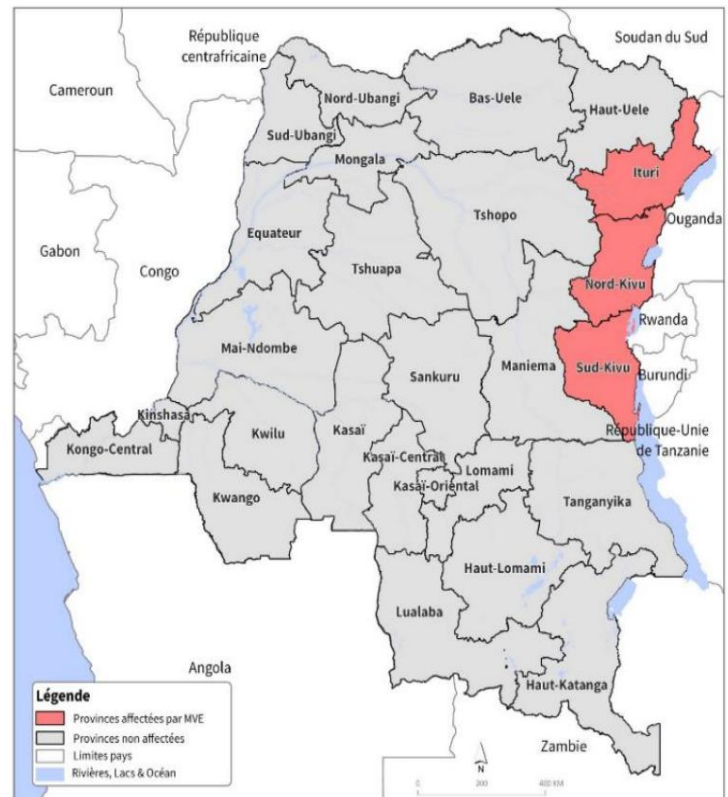
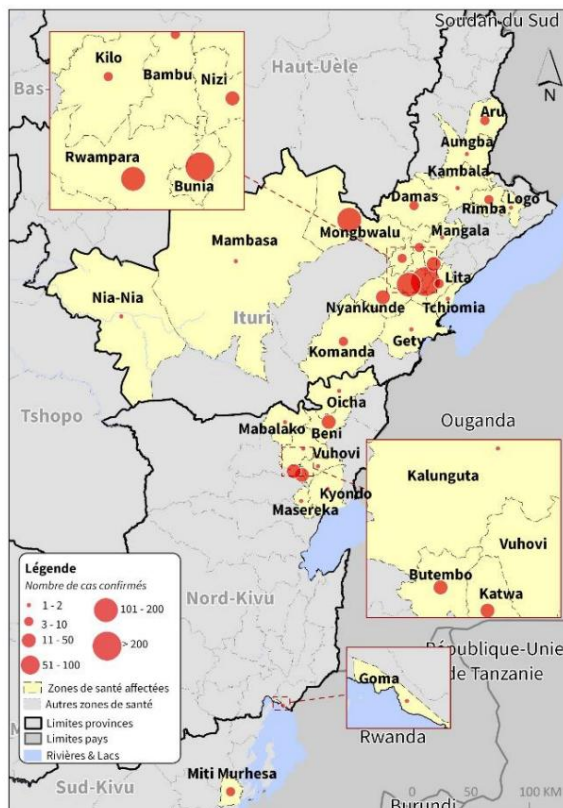
- **South Kivu Province (3 cases) : Miti-Murhesa (3).**

3.3 Time analysis: The number of confirmed cases is increasing week by week, indicating continued transmission of the disease within the community. A rapid geographical expansion of the epidemic is a real possibility if public health measures are not implemented quickly.



Source : DHIS2 Tracker - Riposte MVE, juin 2026. Données incluses dans l'analyse : n cas confirmés = 589.

3.4 Epidemic mapping



Figures 2 & 3. Spatial distribution of confirmed Ebola cases by affected health zone and in the three affected provinces as of June 13, 2026

Table 1. Distribution of confirmed cases and deaths of Bundibugyo Ebola virus disease by health zone and province, Democratic Republic of Congo, as of June 13, 2026.

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (%)
ITURI			
Bunia	212	20	9.4%
Rwampara	149	27	18.1%
Mongbwalu	164	73	44.5%
Nyankunde	38	4	10.5%
Bamboo	7	2	28.6%
Aru	3		33.3%
Kilo	4	11	25.0%
Nizi	11		9.1%
Mangala	5	1	60.0%
Damascus	4	3	0.0%
Aungba	2	01	50.0%
Gety		0	0.0%
Komanda	1	0	0.0%
Lita	6	0	0.0%
Logo	6	0	0.0%
Mambasa	22	1	50.0%
Rimba	3	0	0.0%
Tchomia	2	0	0.0%
Kambala			100.0%
Nai-Nia	11	11	100.0%
Other ZS (undisclosed data)	94	10	10.6%
Ituri subtotal	717	146	20.4%
NORTH KIVU			
Katwa	21	12	57.1%
Blessed	14	10	71.4%
Butembo	18	7	38.9%
Oicha	2	2	100.0%
Kalunguta	1	1	100.0%
Kyondo	2		50.0%
Goma	1	1	0.0%
Masereka	1	0	0.0%
Vuhovi	1		100.0%
Mabalako	1	010	0.0%
North Kivu subtotal	62	34	54.8%
SOUTH KIVU			
Miti-Murhesa			33.3%
South Kivu subtotal			33.3%
TOTAL	3,3782	1 1 181	23.1%

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri:

- Continuation of the DRC-Uganda cross-border meeting on the Ebola virus disease situation on June 12 2026.
- Continued training of MCZ and AC for the Aru pool.

4.2 Epidemiological surveillance

Table 2. Management of epidemiological alerts (24h) as of June 13, 2026

Indicator		Ituri	North Kivu	South Kivu	Total
Alerts raised		152	332	23	507
Alerts investigated		106	332	22	460
Investigation rate		69.7%	100.0%	95.7%	90.7%
Alerts confirmed	Alive	54	25	8	87
	Deceased	30	19	0	49
Total suspected cases for today		84	44	8	136

Based on the data submitted for June 13, 2026, **507 alerts** were raised across the three affected provinces, of which **460 (90.7%)** were investigated. Thus, **136 suspected cases (49 deaths)** were recorded.

Table 3. Contact tracing of confirmed cases as of June 13, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri	5065	2,879	56.8%
North Kivu	1,130	589	52.1%
South Kivu	80	80	100.0%
Total	6,275	3,548	56.5%

- Thorough investigation and exhaustive listing and follow-up of contacts around the confirmed cases of June 13, 2026 in the 17 affected health zones (narratives of confirmed cases, number of contacts listed and followed up per case) • 449 new contacts were listed in 8 health zones in Ituri province and 105 contacts (South Kivu) successfully completed their 21-day follow-up.

Update on checkpoint and point of entry (PoC/PoE) activities

- Activation of 2 PoCs (Old barrier and PoC Ogouro) on the Bunia-Mahagi-Anzida axis.
- Briefing of service providers at the Mahagi and Mambasa PoCs on the minimum package of activities during the emergency room.

Table 4. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC) in Ituri province, as of June 13, 2026

Indicators	Results
Proportion of activated PoCs	72.7% (40/55)
Number of travelers who passed through, % of travelers	47,525
screened, % of travelers who washed their hands, % of travelers made aware of the risks	90.0%
Number of alerts notified	94.0%
Percentage of alerts investigated or bodies swabbed within 24 hours	94.0%
Number of validated alerts	2
% of results available in 24 hours	50.00%
Number of alerts transferred to the CT/CTE or for EDS	2
Number of contacts intercepted today	0%
	0
	0

4.3 Laboratory

- **Ituri:** 139 new samples collected and analyzed (positivity 40.3%; n=56).
- **South Kivu :** 8 samples collected and analyzed, all came back negative.
- **North Kivu :** 42 samples collected, of which 34 were analyzed (Bundibugyo strain); 16 are positive income (47.1%).

4.4 Infection Prevention and Control (IPC)

- **Ituri:** Reception of 3 Land Cruiser vehicles for the EDS and decontamination team (Nyankunde Health Zone); opening of rings around 5 confirmed cases (households and identified SSEs), including 3 in the Bunia Health Zone and 2 in the Rwampara Health Zone with the beginning of contamination, while the IPC kits represent a gap; 11/15 EDS carried out in the Mongbwalu Health Zone, 7/10 EDS in the Bunia Health Zone; 5/5 EDS in the Komanda Health Zone, 4/5 EDS in the Rwampara Health Zone, 4/4 in the Bambu Health Zone; 24 service providers from 5 SSEs in the Bunia Health Zone were briefed on Ebola prevention measures, waste management and other basic IPC concepts; around the confirmed cases, decontamination was organised in ESS (1 in Bunia and 1 in Rwampara), households (28 in Mongbwalu, 2 in Rwampara and 1 in Bunia) and public buildings (1 in Bunia and in Mongbwalu).
- **North Kivu:** Evaluation of 7 ESS in Beni (CH Sifa 65%, UCBC 63.5%, CH beni 61.4%, CS Mabakanga 49%, CM Gracia 41%, Carl Becker 76%, CM La Gazelle 29%) with an average score of 54.9%; support for 59 ESS in the health zones of Goma, Beni, Butembo and Katwa; 61 providers briefed on the different IPC themes in the context of the Ebola outbreak; assessment of the degree of exposure of 2 providers at CH Fepsi, all at high risk; continuation of IPC training of 11 providers from refugee transit camps in North Kivu with technical support from WHO.
- **South Kivu:** Reception in Bukavu and development of the distribution plan of 4600Kg/8,652m3 PPE and PCI inputs to the 15 priority health zones of South Kivu; assessment of PCI needs at the Nyakadaka cantonment site of 10 buildings; theoretical and practical briefing of 65 service providers on standard precautionary measures and decontamination technique; support of the Kahungu health center and briefing of service providers on standard PCI precautionary measures.

4.5 Holistic care

- Five (5) patients (Rwampara: 3, Mongbwalu: 1 and Mambasa: 1) with Ebola virus disease were declared recovered after obtaining negative follow-up tests.
- Monitoring of construction work for nurseries and CTEs (CME, ISTM Nyankunde, HGR Nyankunde, HGR Rwampara and Elykia).
- Monitoring and nutritional care of patients in all CTEs: 470 hot meals were offered (179 suspected, 110 confirmed, 159 PPL and 22 accompanying persons).

Table 5. Patient movement within healthcare facilities (CTE/CT/CI) as of June 13, 2026

Indicator of	Ituri	North Kivu	South Kivu	Together
Patients in Bed (Day -1)	280	36	11	327
Total admissions (24h)	60	6	2	68
Exits (24h)				
Deceased (Suspects/Confessions)	13	0	0	13
No cases / Cured	16	0	6	22
Escapees (Suspects/Confessions)	0	0	0	0
Transferred to the HGR	1	0	0	0
Total outflows	30	0	6	36
Patients in isolation (End of Day)	310	42	7	359
including confirmed (NC+AC)	140	6	1	147
and suspected cases	170	36	6	212
Overall occupancy rate	93.9%	59.2%	21.9%	82.9%

Mental health activities and psychosocial support

Table 6. Key indicators of mental health and psychosocial support activities as of June 13, 2026

KEY INDICATORS	ITURI	SOUTH KIVU	NORTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	4 (80%)	0	ND
Confirmed cases currently being monitored that have benefited from SMSPS interventions	56 (61.5%)	1 (100%)	ND
New suspected cases that benefited from SMSPS interventions	14 (43.8%)	2 (100%)	ND
Suspected cases under follow-up that have received interventions SMSPS	45 (60%)	0	ND
Laboratory results have been announced	14	7	ND
Including positive ones	11	0	ND
Number of children separated in daycare who benefited from SMSPS interventions	0	1 (100%)	ND
Number of children separated in the community who have benefited from interventions SMSPS	5	1	ND
Number of accompanying persons at the CTE who benefited from SMSPS interventions	39	0	ND
Number of PPLs who benefited from SMSPS interventions	7	0	ND
Number of household and community members who benefited from SMSPS interventions	183	0	ND
Number of EDS supported	0	0	ND

4.6 Continuity of services.

- Completion of the evaluation of the 3 ESS in the Komanda health zone.
- Continued implementation of the recovery plans for social and solidarity economy enterprises already assessed.

4.7 Risk communication and community engagement

North Kivu: 419 students from Lumière University of Bujumbura, Goma branch, were sensitized on the signs and methods of prevention in order to improve early detection and notification of suspected cases in academic settings; 366 community health workers and other community actors visited 1,998 households and sensitized 7,986 people through door-to-door activities and reported 18 Ebola alerts and 503 instances of reluctance/refusal/resistance recorded and resolved in the health zones of Goma, Karisimbi, Nyiragongo; 3,153 people were sensitized in public places (parking lots, markets, ports, churches, etc.) in the health zones of Goma, Karisimbi and Nyiragongo; 359 community leaders, community health workers, grassroots administrators, teachers, traditional healers, pastors and pharmacy owners, religious leaders, journalists, etc., were trained on prevention messages, CREC (Community Risk Assessment), community management and feedback, and infodemics; 18 radio stations produced 18 programs and 36 public service announcements with widespread dissemination of messages via these local radio stations on prevention, the signs of Ebola virus disease (EVD), and hand hygiene; 27 taxi drivers were briefed on EVD, motorcycle hygiene, and adherence to EVD preventive measures in the Vighole health district/Katwa health zone; 16 community health workers in the Mitoya health district were briefed on risk communication and community engagement; 35 school principals, cell leaders, and community health center presidents were briefed on EVD, hotel hygiene, and adherence to EVD preventive measures in the Kyangike health district; 15 community leaders in the Vulamba health district were briefed on risk communication and community engagement. 3,578 people made aware of the symptoms of Ebola virus disease, including 1,240 men and 2,158 women: 21 alerts reported by the community.

South Kivu: 4 radio broadcasts on community engagement to break the chain of transmission in the Miti-Murhesa health zone reached approximately 2,500 people; briefing of 98 people (community facilitators, members of local and international organizations) on Ebola virus disease (EVD), community-based infection prevention and control (CBIC) and infodemic management; 1,230 households visited, 4,317 people sensitized, 4 radio stations broadcast prevention messages, 19 leaders engaged in the Miti-Murhesa and Katana health zones; mass awareness-raising of 265 final-year primary school students from Umoja school in the Miti-Murhesa health zone on prevention measures and signs of EVD.

Ituri: Orientation meeting of the 14 opinion leaders and leaders of the groups responsible for supporting the prevention and management of community incidents on the sidelines of their deployment in order to prepare families for public health interventions around cases in hotspots; a total of 586 people (175 men and 411 women) in 5 health zones were reached through 4 organized mass awareness campaigns on several themes (preventive measures, reporting alerts from the animal sector, importance of EDS, importance of contact tracing, etc.); during home visits by 60 community relays, 727 households were visited in the health zones of Bunia and Nyankunde, 6,014 (2,820 men and 3,194 women) people were reached with messages on respecting preventive measures for Ebola and reporting community alerts; training of 15 community relays in the Abelkonzo AS/Mongbwalu ZS on community-based surveillance; 21 broadcasts and 18 airings of spots and messages respectively on general information about Ebola virus disease, dignified and safe burial, testimonies of those who have recovered and preventive measures for Ebola virus disease.

Infodemic Management/Community Feedback : It has become a recurring issue that EDS teams arrive in very small numbers at the burial site and community members are forced to help them without protection at the risk of becoming infected.

Support to other pillars : Surveillance : Support for investigations in the Shari, Ngona, Hoho, Rwampara, Dele, and Sukisa health districts; **IPC:** Preparation of two families for household decontamination (Lengabo health district, Rwampara health zone). Result: Decontamination completed; preparation of the two families for participation in the Health Development Plan (Shari health district, Rwampara health zone). Result: Health Development Plans completed

4.8 Logistics and Security

- Receipt of 164 mattresses, 140 beds intended for ESS.
- Finalization of the CTE/CME-BUNIA works; 8 tents installed will be operational by Monday the 15th June 2026.
- Deployment of 28 vehicles and 2 ambulances for different pillars of the response in order to ensure the team mobility.
- Delivery of motorcycles to the Logo and Aungba ZS.

4.9. Security

- The security situation remains calm but unpredictable in Bunia and surrounding areas.
- Validation of the evaluation form for experts leaving the Ebola outbreak zone.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

- Continued awareness-raising efforts among local communities regarding complaint mechanisms.

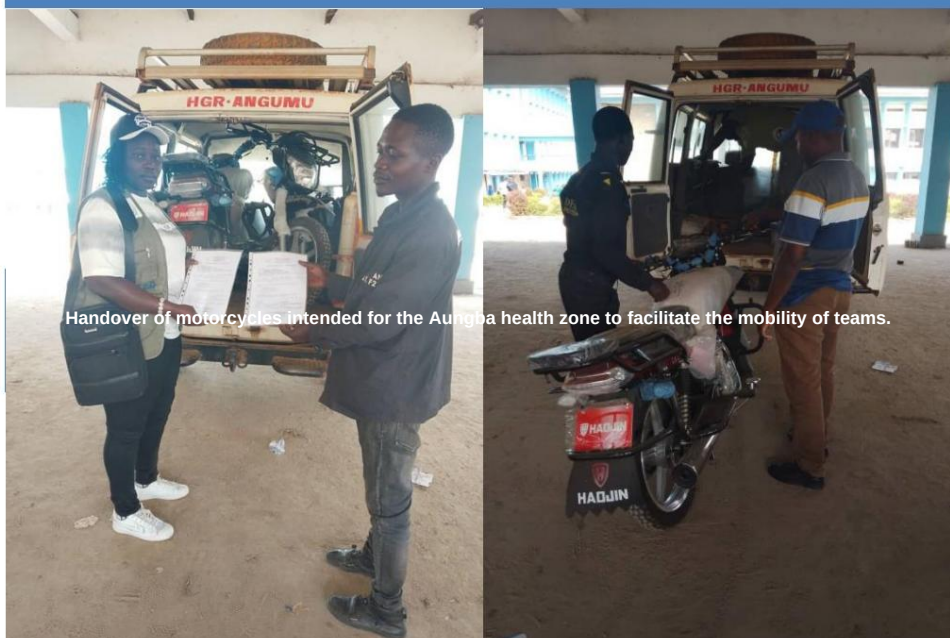
5. MAIN CHALLENGES

Challenge	Impact	Action required
No. 1 Reluctance to perform post-mortem sampling (swabs)	Confirmation delay; underestimation of cases	Intensive community engagement; involvement of religious leaders
2 Insufficient capacity in standardized CTEs despite the opening of Rwampara	Potential saturation of existing CTEs	Acceleration of the construction of planned CTEs
3 Weakness in contact tracing (overall rate 56.5%, target 95%)	Interruption of compromised transmission chains	Training, deployment and motivation of community relays
4 Supply shortage Essential Generic Medicines (MEG) in targeted HGRs	Weakening of overall care	A plea for MEG funding
5 Insufficient PCI equipment (PPE, DLM, chlorine) in North Kivu	Nosocomial risk	Urgent supplies
6 Low increase in alerts in the 3 provinces	Delay in detection of case	Strengthening community surveillance
7 Insufficient financial resources for all pillars	Slowdown in operations	Resource mobilization (gap of USD 21.5 million)

6. PERSPECTIVES AND PRIORITY ACTIONS (24-72h)

Priority Action		Responsible pillar	Due date
	Training of service providers on the use of the RADIONE PCR	Laboratory	Day 2
1 2	Briefing of service providers on Ebola virus disease (EVD) and tuberculosis control (TBC) surveillance in affected and high-risk areas	Monitoring/CREC Day 3	
3	Briefing of community agents on Ebola surveillance and the SBC (Ituri province)	Monitoring/CREC Day 3	
4	Supervised support from the Director General of INSP in the technical pillars	Coordination	Continuous
5	Catch-up in contact tracing in Ituri remains weak.	Monitoring	Day 7
6	Strengthening infection prevention and control (IPC) measures and staff training in Nyankunde and other nosocomial areas	PCI	Day 2
7	Deployment of a standardized daily dashboard with KPIs per pillar	Coordination	Day 3
8	Plan for catching up on samples awaiting analysis in North Kivu Laboratory		Day 2
9	North Kivu: discharge of the recovered case in Goma (Day 1), response to security incidents (ADF incursion), IPC training at Virunga General Hospital, CREC extension to households		Day 2
10	South Kivu: provision of 2 additional vehicles, installation of an internet kit in Miti-Murhesa, communication credits for the IT/ECZS		Day 2

Handover of motorcycles to the Aungba health zone to facilitate Ebola response activities, Ituri province.



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